



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Youth Unit: safe ward environment

Reference #: FSH-MENH-POL-0011

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Youth Unit (MHB)	All staff

1.Introduction

To ensure a proactive approach to preventing patient harm and managing patient risks, the following policy and procedure documents the processes that are in place to maintain a safe environment and minimise risk on the Youth Unit including:

- Patient access to areas which may provide means of self-harm.
- Management of unsafe/potentially harmful items.
- Maintaining safe staffing levels
- Separation of the ward environment.
- Segregation of children from adults

2.Terminology

Ligature Points	Any feature in an environment which could be used to support a noose or other strangulation device for the purpose of attempting suicide. Ligature points may include: Doors, windows, cupboards, wardrobes, sinks, bath / sink taps, exposed pipes, fire extinguishers, curtains blinds / rails, ceiling extractor fans, radiators, fire alarms, toilet cistern / toilet roll / soap, paper towel dispensers, light fittings, Shower head / control shower cubicle doors / curtain rails.
Unsafe items	Unsafe items may include sharp instruments, knives, razors, glassware, pins, spray deodorants, hair ties, plastic bags, matches, lighters, belts, ties, shoelaces, electrical cables / cords, gym equipment (e.g. dumbbells).
Child	Under the Mental Health Act 2014 (MHA2014) (Part 2 Division 1 Section 4) a child is defined as a person who is under 18 years of age.

3. Policy

- Ligature points will be, where possible, removed or covered. If it is not possible to remove or cover all potential ligature points, risk mitigation and management strategies must be in place.
- Patients are only allowed access to areas of the Youth Unit where known ligature risk points have been identified when staff member/s are available to supervise. These areas include:
 - Central family room
 - Comfort/Quiet room
 - Interview rooms
 - Courtyard 5
 - Gymnasium
 - Activities of daily living (ADL) kitchen

All staff must be made aware of any blind spots within the unit. The use of monitoring equipment or mirrors to reduce this risk must be considered.

- To ensure the safety of patients and staff on Youth Unit access to unsafe/potentially harmful items must be restricted.
- Access, reconciliation, and storage of unsafe/potentially harmful items will be managed by Youth Unit staff.
- The decision to divide the ward into two discrete units (eight beds and six beds) will only be made when it is considered it provides the safest possible environment for the patients on the ward and other strategies including specialising individual's patients have not adequately mitigated risk.
 - A staff base must be located within each section of the ward when divided.
- Clinical staff will document a summary of the actions to be taken to ensure the safety of admitted patients under the age of 18 years and how age specific care will be delivered. A copy will be provided to the child's parent/guardian, the Office of the Chief Psychiatrist and filed in the Digital Medical Record (DMR).

4. Procedure

4.1. Admission to ward

- On admission to the Youth Unit:
 - the patient will be advised that their bedroom is their own private space and other patients, and visitors are not allowed in their room.

- staff will ask to examine any belongings brought in including items in their clothing to prevent unsafe items being carried onto the unit. Refer to the SMSH [Patient search and identification of dangerous items](#) policy.
- patients under 18 years will have a MHA s303 Segregation of children from adult inpatients notification form will be completed as per the [MHA s303 Segregation of children from adult inpatients](#)
- Involuntary mental health inpatients, so far as reasonably practicable, are to be provided with a secure place to store their personal items / property.
- Where items are removed from the patient the clinical rationale for decision making e.g. individual risk, ward environment must be detailed in the patient's medical record.
- Patient's own medications (prescribed and over the counter) must be stored in the clean utility room.
- A review of the patients own medications will occur during the admission by the treating team and pharmacist wherever possible. A decision will be made regarding those medications that are safe to return to the patient on discharge.
- Where review of medications has not occurred, and staff have concerns regarding returning patients own medications at discharge they will discuss with the Treating Team (in hours) or After Hours Registrar / Consultant.
- Items considered to present a risk must be stored in the patient storeroom. Items that are deemed to be safe can be stored in the patient's bedside draw.
- All patient property brought into the mental health unit will be documented on the Patient Property form. A copy of the form must be given to the patient.
- If visitors bring in additional belongings and other items another Patient Property form needs to be completed and added to the original.

4.2. Risk assessment and management

- All patients will undergo clinical risk assessment on admission and at regular reviews. This will include the risk to patient of unsafe / potentially harmful items. Refer to the FSFHG Clinical Risk Assessment and Management in Mental Health policy.
- The risk assessment will inform the individual patient's observation requirements which will be documented on their Treatment Support and Discharge Plan (TSDP) and DMR.

4.3. Ward environment and Patient Observation

- Environmental checks of the ward and courtyards by the Shift Coordinator (or delegate) will be conducted at the commencement of each shift and documented on the environmental checklist
- Patients will be accompanied at all times in areas of the ward that contain unsafe/potentially harmful items e.g. family and interview rooms,

comfort/quiet room, gymnasium, kitchen and Courtyard 5 and these areas will be locked when not in use.

- All equipment in these areas will be reconciled by staff daily or after each therapy session. Equipment will then be locked in the provided cupboards after use.
- An Activity of Daily Living (ADL) kitchen area is provided on the Unit. The ADL kitchen is only for patients and no additional equipment should be brought into this area e.g. china plates, glasses.
- Any kitchen equipment required for functional assessment or groups that may be unsafe / potentially harmful items (such as sharp knives) is provided by staff as required and patients must be supervised during use.
- After use, the equipment will be collected and reconciled by staff. Sharps are signed for in the sharps register and locked in the drawer provided between uses.

4.4. Ward separation

- Separating the ward has implications for staffing, capacity and access and is only indicated when:
 - patient acuity is high
 - there are concerns for the sexual safety of patients
 - age groups are incompatible
 - infection outbreak has occurred
- The decision to separate the ward will be made collaboratively between:

In hours	After hours
Head of Service/delegate	On call Consultant Psychiatrist / Registrar
Nurse Director	Hospital Out of Hours Team (HOOT)
Nurse Unit Manager (NUM) / Clinical Nurse Specialist (CNS)	Shift Coordinator

- Separation of the Youth Unit will be communicated to:
 - patients/carers
 - Youth Unit multi-disciplinary team
 - Service Director/Executive On Call
 - Capacity and Access – 24 hour collated report
 - Housekeeping for meal delivery and bed cleaning
- In the event of ward separation for clinical reasons a minimum of two Nursing staff must always be present in each section of the ward.
- Where it is safe to do so, and agreed to by the Shift Coordinator and treating Consultant Psychiatrist the reconfigured ward will allow access to:

- Comfort/Quiet Room if clinically indicated
 - Gymnasium when appropriate
 - Courtyard 5 for rooms 9 to 16
 - Courtyard 6 for rooms 17 to 22
- The ongoing requirement to separate the ward will be discussed at the morning multidisciplinary team meeting and reviewed each shift by:
 - Head of Service/Consultant on call
 - NUM/HOOT
 - Nurse Director
 - Shift Coordinator
- Return to the usual bed configuration will occur as soon as the precipitating circumstances are resolved.
- The Shift Coordinator will complete the Youth Unit Ward Division Notification form as soon as possible and forward to the NUM.

4.5. Visitors

- Items brought into the unit by visitors for patients must be inspected by the patient's Nurse to ensure there are no items that may pose a risk on the ward and will be documented on the Patient Property form. Refer to the FSFHG Visiting in Mental Health Inpatient areas policy.
- Lockers will be provided to store visitor belongings if required

5. Compliance/performance monitoring

A database of all instances of ward separation, reasons and duration will be maintained by administration staff and monitored by the Service 5 Nurse Director.

The NUM is responsible for reviewing each occurrence of ward separation with key stakeholders to evaluate the effectiveness of the risk mitigation strategy.

An annual Access to means of self-harm audit will be completed and actioned by the CNS and Service 5 Safety Quality and Risk team.

6. Related policy documents

SMHS:

- [Patient search and identification of dangerous items](#)
- [Rights of patients in mental health](#)

FSFHG:

- Medication Administration (FSFH-HW-POL-0022)

- Patient Observation – Specialising and Close Observation (FSFH-MENH-POL0023) Clinical risk assessment and management (FSFH-HW-POL-0017)
- Courtyard access and observation – Mental Health (FSFH-MENH-POL-0005)
- CCTV monitoring in mental health clinical areas (FSFH-MENH-POL-0011)
- Sensory rooms and sensory equipment: safe and therapeutic use (FSFH-MENH-GUI-0006)
- Sexual safety in Mental Health (FSFH-MENH-POL-0012)
- Visiting in Mental Health Inpatient areas (FSFH-MENH-POL-0016)
- Code Black in the Mental Health Unit (FSH-MENH-POL-0014)
- Nursing staff escalation process – Mental Health (FSH-MENH-GUI-0005)

7. Related Standards

NSQHS Standards:

- Clinical Governance
- Comprehensive Care
- Communicating for Safety

The Chief Psychiatrists Standard's for Clinical Care:

- Care Planning
- Risk assessment and Management

8. Related documents

[The Mental Health Act 2014](#)

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director – Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	11/2015	ND	Clinical Commissioning	Policy Committee	11/2018
2	10/2020	CNS Youth Unit	S5 SQR Committee	FSFHG Policy Committee	10/2023
3	08/2023	CNS Youth Unit		S5 SQR Committee	09/2027